

File Information		Artwork Type [PLEASE TICK ✓]		Sign / Stamp	
Product : T ASEPTONE SYRUP 5MG/1ML [FRONT] SAP No. : Version : RASUB/V2 Date : 22 July 2024		☐ Unit Box ☐ Outer Box ☐ Label ☒ Leaflet ☐ Fix-A-Form ☐ Others : _____ ☐ Aluminium Foil ☐ Sachet			
Document Category [PLEASE TICK ✓]		Pharmacode / QR Code / Others			
☒ RA Submission Artwork ☐ Internal Artwork Circulation ☐ Others : _____ ☐ NPRA Approval Copy ☐ Previous Approved Artwork ☐ Approval Copy Artwork ☐ Draft Artwork					
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METHADONE HCL

ASEPTONE SYRUP 5 MG / 1 ML

DESCRIPTION:

A red coloured syrup.

COMPOSITION:

Each ml contains Methadone Hydrochloride 5mg.

PHARMACODYNAMICS:

Methadone HCl is a primarily μ opioid agonist. It binds with stereospecific receptors at many sites within the central nervous system (CNS). The most prominent involve the CNS and organs composed of smooth muscle.

PHARMACOKINETICS:

Protein binding : High
Half life : 15 – 25 hours; increases with repeated administration.
Onset of action : Oral: 30 – 60 minutes
Time to peak effect : Oral: 1.5 – 2 hours
Duration of action (in non-tolerant patients, may increase considerably with chronic use because of accumulation of methadone or active metabolites; may then decrease as tolerance develops during chronic therapy): Oral: 4 – 6 hours
Elimination: Primarily renal (rate increased in acidic urine); also some biliary elimination.
The methadone abstinence syndrome, although qualitatively similar to that of morphine, differs in that the onset is slower, the course is more prolonged, and the symptoms are less severe. When administered orally, methadone is approximately one-half as potent as when given parenterally.

INDICATIONS: It is indicated in the detoxification treatment of narcotic addiction.

RECOMMENDED DOSAGE:

For detoxification Treatment:

THE DRUG SHALL BE ADMINISTERED DAILY UNDER CLOSE SUPERVISION AS FOLLOWS:

A detoxification treatment course shall not exceed 21 days and may not be repeated earlier than four weeks after completion of the preceding course. In detoxification, the patient may receive methadone when there are significant symptoms of withdrawal. The dosage schedules indicated below are recommended but could be varied in accordance with clinical judgment. Initially, a single oral dose of 15 to 20mg of methadone will often be sufficient to suppress withdrawal symptoms. Additional methadone may be provided if withdrawal symptoms are not suppressed or if symptoms reappear. When patients are physically dependence on high doses, it may be necessary to exceed these levels. 40mg per day in single or divided doses will usually constitute an adequate stabilizing dosage level. Stabilization can be continued for 2 to 3 days, and then the amount of methadone normally will be gradually decreased. The rate at which methadone is decreased will be determined separately with each patient. The dose of methadone can be decreased on a daily basis or at 2 days intervals, but the amount of intake shall always be sufficient to keep withdrawal symptoms at a tolerable level. In hospitalized patient, a daily reduction of 20 percent of the total daily dose may be tolerated and may cause little discomfort. In ambulatory patients, a somewhat slower schedule may be needed. If methadone is administered for more than three weeks, the procedure is considered to have progressed from detoxification or treatment of the acute withdrawal syndrome to maintain treatment, even though the goal and intent will be eventual total withdrawal. Patients younger than 18 months of age may be admitted to Methadone maintenance programs only after 2 documented attempts at short term (up to 30 days) detoxification or drug-free treatment have failed. A 1-week waiting period is required between a detoxification attempt and admission to a methadone maintenance program.

For Maintenance Treatment:

In maintenance treatment, the initial dosage of methadone should control the abstinence symptoms that follow withdrawal of narcotic drugs but should not be great as to cause sedation, respiratory depression, or other effects of acute intoxication. It is important that the initial dosage be adjusted on an individual basis to the narcotic tolerance of the new patient. If such a patient has been a heavy user of heroin up to the day of admission, he / she may be given 20mg 4 to 8 hours later or 40mg in a single oral dose. If the patient enters treatment with little or no narcotic tolerance (eg. if he / she has recently been released from jail or other confinement), the initial dosage may be one-half these quantities. When there is any doubt, the smaller dose should be used initially. The patient should then be kept under observation, and, if symptoms of abstinence are distressing, additional 10mg doses may be administered as needed. Subsequently, the dosage should be adjusted individually, as tolerated and required, up to a level of 120mg daily. The patient will initially ingest the drug under observation daily, or at least 6 days a week, for the first 3 months. After demonstrating satisfactory adherence to the program regulation for at least 3 months, the patient may permitted to reduce to 3 times weekly the occasions when he / she must ingest the drug under observation. A regular review of dosage level should be made by the responsible physician, with careful consideration given to reduction of dosage as indicated on an individual basis. A new dosage level is only a test level until stability is achieved.
Special Consideration for a Pregnant Patient
Caution shall be taken in the maintenance treatment of pregnant patients. Dosage level should be kept as low as possible if continued methadone treatment is deemed necessary.

ROUTE OF ADMINISTRATION: Oral

CONTRAINDICATIONS:

Respiratory depression, obstructive airways disease, ulcerative colitis, severe hepatic impairment. Concurrent administration with mono-amine oxidase inhibitors, or within 2 weeks of discontinuation of treatment with them. Obstetric use is not recommended because in labour the prolonged duration of action increases the risk of neonatal depression. Hypersensitivity to methadone. Methadone is not suitable for children. Use during an acute asthma attack is not advisable. Avoid in acute alcoholism and where risk of paralytic ileus, also avoid in raised intracranial pressure or head injury.

WARNING & PRECAUTIONS:

This preparation may be habit forming on prolonged use. Avoid alcoholic beverages. Aseptone syrup is available with sodium saccharin as a sweetening agent. Phenylketonurics are warned that this product, in some forms, contains phenylalanine.

Drug dependence- Methadone can produce drug dependence of the morphine type and, therefore, has the potential for being abused. Psychic dependence, physical dependence, and tolerance may develop upon repeated administration of methadone, and it should be prescribed and administered with the same degree of caution appropriate to the use of morphine.

Anxiety: Since methadone as used by tolerant subjects at a constant maintenance dosage, is not a tranquilizer, patients who are maintained on this drug will react to life problems and

stresses with the same symptoms of anxiety as do other individuals. The physician should not confuse such symptoms with those of narcotic abstinence and should not attempt to treat anxiety by increasing the dosage of methadone. The action of methadone in maintenance treatment is limited to the control of narcotic symptoms and is ineffective for relief of general anxiety.

Head injury and Increased Intracranial Pressure:

The respiratory depressant effects of methadone and its capacity to elevate cerebrospinal-fluid pressure may be markedly exaggerated in the presence of increased intracranial pressure. Furthermore, narcotics produce side effects that may obscure the clinical course of patients with head injuries. In such patients, methadone must be used with caution and only if it is deemed essential

Asthma and other Respiratory Condition: Methadone should be used with caution in patient having an acute asthmatic attack, in those with chronic obstructive pulmonary disease or cor pulmonale, and in individuals with a substantial decreased respiratory reserve, preexisting respiratory depression, hypoxia, or hypercapnia. In such patients, even usual therapeutic doses of narcotics may decrease respiratory drive while simultaneously increasing airway resistance to the point of apnea.

Hypotensive Effects: The administration of methadone may result in severe hypotension in an individual whose ability to maintain his blood pressure has already been compromised by depleted blood volume or concurrent administration of such drugs as the phenothiazines or certain anesthetics.

Use in Ambulatory Patients: Methadone may impair the mental and/or physical abilities required for the performance of potentially hazardous task, such as driving a car or operating machinery. The patient should be cautioned accordingly. Methadone like others narcotics, may produce orthostatic hypotension in ambulatory patient.

Special-Risk Patients: Methadone should be given with caution and the initial dose should be reduced in certain patients, such as the elderly or debilitated and those with severe impairment of hepatic or renal function, hypothyroidism, Addison's disease, prostatic hypertrophy, or urethral stricture. Not recommended for children.

Effects on the Ability to Drive or Operate Machines: This may be severely affected during and after treatment with Methadone. The time which such activities may be safely resumed is extremely patient dependent and must be decided by the physician.

Risks from Concomitant Use with Benzodiazepines

Profound sedation, respiratory depression, coma, and death may result from the concomitant use of Aseptone Syrup 5mg/mL with benzodiazepines. Observational studies have demonstrated that concomitant use of opioids and benzodiazepines increases the risk of drug-related mortality compared to use of opioids alone. Because of these risks, reserve concomitant prescribing of these drugs for use in patients for whom alternative treatment options are inadequate.If the decision is made to newly prescribe a benzodiazepine and an opioid together, prescribe the lowest effective dosages and minimum durations of concomitant use. If the decision is made to prescribe an opioid in a patient already taking a benzodiazepine, prescribe a lower initial dose of the opioid, and titrate based on clinical response.

Follow patients closely for signs and symptoms of respiratory depression and sedation. Advise both patients and caregivers about the risks of respiratory depression and sedation when Aseptone Syrup 5mg/mL is used with benzodiazepines. Advise patients not to drive or operate heavy machinery until the effects of concomitant use of the benzodiazepines have been determined. Screen patients for risk of substance use disorders, including opioid abuse and misuse, and warn them of the risk for overdose and death associated with the use of benzodiazepines (See Drug Interactions).

Serotonin Syndrome with Concomitant Use of Serotonergic Drugs

Cases of serotonin syndrome, a potentially life-threatening condition, have been reported during concurrent use of Aseptone Syrup with serotonergic drugs (See Interactions with Other Medicaments). This may occur within the recommended dosage range. Serotonin syndrome symptoms may include mental-status changes (e.g. agitation, hallucinations, coma), autonomic instability (e.g. tachycardia, labile blood pressure, hyperthermia), neuromuscular aberrations (e.g. hyperreflexia, incoordination) and/or gastrointestinal symptoms (e.g. nausea, vomiting, diarrhoea) and can be fatal (See Interactions with Other Medicaments). The onset of symptoms generally occurs within several hours to a few days of concomitant use, but may occur later than that. Discontinue Aseptone Syrup if serotonin syndrome is suspected.

Adrenal Insufficiency

Cases of adrenal insufficiency have been reported with opioids use, more often following greater than one month of use. Presentation of adrenal insufficiency may include non-specific symptoms and signs including nausea, vomiting, decreased appetite, fatigue, weakness, dizziness, and low blood pressure. If adrenal insufficiency is suspected, confirm the diagnosis with diagnostic testing as soon as possible. If adrenal insufficiency is diagnosed, treat with physiologic replacement dosing of corticosteroids. Wean the patient off of the opioid to allow adrenal function to recover and continue corticosteroids treatment until adrenal function recovers. Other opioids may be tried as some cases reported use of a different opioid without recurrence of adrenal insufficiency. The information available does not identify any particular opioids as being more likely to be associated with adrenal insufficiency.

Sexual Function/Reproduction

Long term use of opioids may be associated with decreased sex hormone levels and symptoms such as low libido, erectile dysfunction, or infertility (See Postmarketing Experience)

INTERACTION WITH OTHER MEDICATIONS

Other Central Nervous System Depressants: Methadone should be used with caution and in reduced dosage in patient who are concurrently receiving other narcotic analgesics, phenothiazines, other tranquilizers, sedative-hypnotics, tricyclic antidepressants, and other C.N.S. depressants (including alcohol). Respiratory depression, hypotension, and profound sedation or coma may result.

Pentazocine: Patient who are addicted to heroin or who are on the methadone maintenance program may experience withdrawal symptoms when given pentazocine.

Rifampin: The concurrent administration of rifampin may possibly reduce the blood concentration of methadone to a degree sufficient to produce withdrawal symptoms. The mechanism by which rifampin may decrease blood concentration of methadone is not fully understood although enhanced microomal drug-metabolized enzymes may influence drug disposition.

Monoamine Oxidase (MAO) Inhibitors: May prolong and enhance the respiratory depressant effects of methadone. Opioids and MAOIs is used together may cause fatal hypotension and coma.

Meperidine: Therapeutic doses of meperidine have precipitated severe reaction in patients concurrently receiving monoamine oxidase inhibitors or those who have received such agents within 14 days. Similar reactions thus far have not been reported with methadone; but if the use of methadone is necessary in such patients, a sensitivity test should be performed in which repeated small incremental doses are administered over the course of several hours while the patient's condition and vital signs are under careful observation.

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Naloxone antagonises the analgesic, CNS and respiratory depressant effects of Methadone.

Naltrexone: Administration of naltrexone to a patient addicted to methadone will rapidly precipitate long term withdrawal symptoms.

Antivirals: Methadone increases plasma concentration of Zidovudine.

Benzodiazepines

Due to additive pharmacologic effect, the concomitant use of opioids with benzodiazepines increases the risk of respiratory depression, profound sedation, coma and death. The concomitant use of opioids and benzodiazepines increases the risk of respiratory depression because of actions at different receptor sites in the central nervous system that control respiration. Opioids interact primarily at μ -receptors, and benzodiazepines interact at GABA_A sites. When opioids and benzodiazepines are combined, the potential for benzodiazepines to significantly worsen opioid-related respiratory depression exists. Reserve concomitant prescribing of these drugs for use in patients for whom alternative treatment options are inadequate (see Warnings and Precautions). Limit dosage and duration of concomitant use of benzodiazepines and opioids, and follow patients closely for respiratory depression and sedation.

Serotonergic Drugs

The concomitant use of opioids with other drugs that affect the serotonergic neurotransmitter system has resulted in serotonin syndrome. If concomitant use is warranted, carefully observe the patient, particularly during treatment initiation and dose adjustment. Discontinue Aseptone Syrup if serotonin syndrome is suspected. Examples of serotonergic drugs are selective serotonin reuptake inhibitors (SSRIs), serotonin and norepinephrine reuptake inhibitors (SNRIs), tricyclic antidepressants (TCAs), triptans, 5-HT₃ receptor antagonists, drugs that effects the serotonin neurotransmitter system (e.g. mirtazapine, trazodone, tramadol), monoamine oxidase (MAO) inhibitors (those intended to treat psychiatric disorders and also others, such as linezolid and intravenous methylene blue) (See Warnings and Precautions).

USE IN PREGNANCY & LACTATION :

Use in Pregnancy: Safe use in pregnancy has not been established in relation to possible adverse effects on fetal development. It should not be used in pregnancy, because in pregnancy, the prolonged duration of action increases the risk of neonatal depression. The administration of methadone or other narcotics may obscure the diagnosis or clinical course of patients with acute abdominal condition.

Use in Lactation: From theoretical considerations, Methadone is likely to be excreted in breast milk. Risk-benefit must be considered when Methadone is administered to a nursing mother in a Methadone maintenance program because use of maintenance dose may cause physical dependence in the infant.

SIDE EFFECTS

THE MAJOR HAZARD OF METHADONE IS RESPIRATORY DEPRESSION AND TO A LESSER DEGREE, CIRCULATORY DEPRESSION, RESPIRATORY ARREST, SHOCK AND CARDIAC ARREST HAVE OCCURRED.

The most frequent observed:

Lightheadedness, dizziness, sedation, nausea, vomiting and sweating. These effects seem to be more prominent in ambulatory patients and in those who are not suffering severe chronic pain. In such individuals, lower doses are advisable. Some adverse reaction may be alleviated in the ambulatory patient if he lies down.

Other adverse reaction include:

Post-marketing Experience:

Metabolic and nutritional disorders

Frequency 'not known': hypoglycaemia

Central Nervous System: Euphoria, Dysphoria, weakness, headache, insomnia, agitation, disorientation, and visual disturbances, increase in intracranial pressure.

Gastrointestinal: Dry mouth, anorexia, constipation, biliary tract spasm.

Cardiovascular: Flushing of the face, bradycardia, palpitation, faintness, syncope, and hypotension.

Genito-Urinary: Urinary retention or hesitancy, antidiuretic effect, and reduced libido and/or potency.

Allergic: Pruritus, urticaria, other skin rashes, edema, and rarely hemorrhagic urticaria.

Maintenance on a Stabilized Dose:

During prolonged administration of methadone, as in a methadone maintenance treatment program, there is a gradual yet progressive disappearance of side effects over a period of several weeks. However, constipation and sweating often persist.

Postmarketing Experience

Serotonin syndrome (See Warnings and Precautions)

Adrenal insufficiency (See Warnings and Precautions)

Androgen deficiency: Cases of androgen deficiency have occurred with chronic use of opioids. Chronic use of opioids may influence the hypothalamic-pituitary-gonadal axis, leading to androgen deficiency that may manifest as low libido, impotence, erectile dysfunction, amenorrhea, or infertility. The causal role of opioids in the clinical syndrome of hypogonadism is unknown because the various medical, physical, lifestyle, and psychological stressors that may influence gonadal hormone levels have not been adequately controlled for in studies conducted to date.

Patients presenting with symptoms of androgen deficiency should undergo laboratory evaluation.

Infertility: Chronic use of opioids may cause reduced fertility in females and males of reproductive potential. It is not known these effects on fertility are reversible.

SYMPTOMS AND TREATMENT OF OVERDOSE

Symptoms: Serious overdosage of methadone is characterized by respiratory depression (a decrease in respiratory rate and / or tidal volume, Cheyne-stokes respiration, cyanosis), extreme somnolence progressing to stupor or coma, maximally constricted pupils, skeletal muscle flaccidity, cold and clammy skin, and sometimes,bradycardia and hypotension. In severe overdosage, particularly by the intravenous route, apnea, circulatory collapse,cardiac arrest, and death may occur.

Treatment: Primary attention should be given to the reestablishment of adequate respiratory exchange through provision of a patent airway and institution of assisted controlled ventilation. If a non-tolerant person, especially a child, take a large dose of methadone, effective narcotic antagonist are available to counter-act the potentially lethal respiratory depression.

THE PHYSICIAN MUST REMEMBER, HOWEVER, THAT METHADONE IS A LONG-ACTING DEPRESSANT (36 TO 48 HOURS), WHEREAS THE ANTAGONIST ACT FOR MUCH SHORTER PERIODS (1 TO 3 HOURS).

The patient must therefore be monitored continuously for recurrence of respiratory depression and treated repeatedly with the narcotic antagonist as needed. If the diagnosis is correct and respiratory depression is due only to overdosage of methadone, the use of respiratory stimulant is not indicated. An antagonist should not be administered in the absence of clinically significant respiratory or cardiovascular depression. Intravenously administered narcotic antagonist, naloxone hydrochloride, nalorphine hydrochloride or levallorphan tartrate are the drug of choice to reverse sign of intoxicification. These agent should be given repeatedly until the patient's status remains satisfactory. The hazard that the narcotic antagonist will further

depress respiration is less likely with the use of naloxone. Oxygen, intravenous fluids, vasopressors, and other supportive measures should be employed as indicated.

NOTE: IN AN INDIVIDUAL PHYSICALLY DEPENDENT ON NARCOTICS, THE ADMINISTRATION OF THE USUAL DOSE OF A NARCOTIC ANTAGONIST WILL PRECIPITATE AN ACUTE WITHDRAWAL SYNDROME. THE SEVERITY OF THIS SYNDROME WILL DEPEND ON THE DEGREE OF PHYSICAL DEPENDENCE AND THE DOSE THE ANTAGONIST ADMINISTERED. THE USE OF A NARCOTIC ANTAGONIST IN SUCH PERSON SHOULD BE AVOIDED IF POSSIBLE. IF IT MUST BE USED TO TREAT SERIOUS RESPIRATORY DEPRESSION IN THE PHYSICALLY DEPENDENT PATIENT, THE ANTAGONIST SHOULD BE ADMINISTERED WITH EXTREME CARE AND WITH SMALLER THAN USUAL DOSE OF THE ANTAGONIST.

STORAGE CONDITIONS:

Store below 30°C. Keep out of reach of children.

Jauhkan daripada kanak-kanak.

PACK SIZE: 60ml

SHELF LIFE: Please refer to outer package.

PRODUCT REGISTRATION HOLDER:

DUOPHARMA MANUFACTURING (BANGI) SDN. BHD.

Lot No. 2, 4, 6, 8 & 10, Jalan P/7,
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